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EDITORIAL

Values, Encounter, and the Limits of What Can Be Done

On the Focus of the Second Issue

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ABSTRACT

The second issue of the first volume of the Journal of Meaning-Centered Psychology shifts its focus from the methodological questions of the first issue toward therapeutic and counseling practice. Under the theme “Meaning, values, and existential challenges in psychotherapeutic practice,” it brings together papers that relate central concepts of logotherapy to contemporary treatments and settings of care. A theoretical paper compares the concept of values in acceptance and commitment therapy with Viktor E. Frankl's categories of values. A review describes meaning-centered psychotherapy in palliative care and appraises its evidence base. A contribution on practice and teaching outlines principles of logotherapeutically informed counseling for burnout and crises of meaning. A book review reassesses Frankl's account “Man's Search for Meaning” from a contemporary perspective. The editorial sketches the common point of reference of these papers: the question of how psychotherapy can deal with what cannot be changed, and which role values and meaning play in this respect. It also recalls the journal's editorial principles, in particular the clear labeling of didactic case vignettes as constructed teaching examples.

Keywords: values · logotherapy · psychotherapeutic practice · palliative care · editorial

From the State of Research to Practice

The first issue of this journal was devoted to the question of how logotherapy can be situated in the context of contemporary psychotherapy research. The present second issue turns more strongly to therapeutic and counseling practice. Its focus is “Meaning, Values, and Existential Challenges in Psychotherapeutic Practice.” This change of perspective does not signify a departure from the standard of scientific rigor; rather, it responds to the observation that conceptual imprecision can have particularly far-reaching consequences in practice.

A unifying motif of the contributions is the question of how psychotherapy can deal with what cannot be changed. In addition to creative values and experiential values, Frankl (1946) described attitudinal values, which are realized in a person’s stance toward unalterable suffering. Yalom (1980) named death, freedom, isolation, and meaninglessness as the ultimate concerns of existence that every psychotherapy must sooner or later confront.

For psychotherapeutic practice, this question is of immediate significance. Many treatment approaches are geared toward bringing about change: symptoms are to decrease, dysfunctional patterns to dissolve, capacities to grow. Yet not everything that burdens people can be changed. Chronic and life-limiting illnesses, losses, irreversible life decisions, or structural conditions of the working world set limits to what can be achieved therapeutically. It is precisely at these limits that the question of meaning and values gains weight.

The Contributions to This Issue

The Cognitive Behavioral Therapy and Contextual Approaches unit compares the concept of values in Acceptance and Commitment Therapy (Hayes et al., 2012) with Frankl’s categories of values and asks where genuine convergences exist and where there are merely family resemblances. The Psycho-Oncology and Palliative Psychology unit presents a review of Meaning-Centered Psychotherapy, which is among

the most carefully investigated meaning-centered interventions (Breitbart et al., 2015).

The contribution from practice and teaching by the Counseling Psychology unit develops the main features of logotherapeutically oriented counseling for burnout and crises of meaning. It works with a teaching vignette explicitly identified as constructed and carefully distinguishes counseling from the treatment of mental disorders. The issue concludes with a review of Frankl's account *… trotzdem Ja zum Leben sagen* (Man's Search for Meaning), which asks about the enduring significance, but also the limitations, of this widely read book.

The contributions show in different ways that engaging with these limits need not mean resignation. It does, however, require an attitude that neither offers comfort prematurely nor glorifies suffering as a means of finding meaning. Frankl himself emphasized that avoidable suffering is to be eliminated and that only unavoidable suffering offers an opportunity to realize attitudinal values (Frankl, 1946).

Outlook

With this issue, the first volume of the journal comes to a close. The editors thank the participating units for their contributions and for their willingness to commit to the journal's principles of integrity: cautious reporting of empirical findings, transparent identification of theoretical premises and constructed case examples, and consistent orientation toward verifiable sources. We look forward to continuing the discussion in the coming volume.

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THEORETICAL ARTICLE

Values in Acceptance and Commitment Therapy and Frankl's Categories of Values

Convergences and Differences Between Two Conceptions of Values

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ABSTRACT

Values occupy a central position in both acceptance and commitment therapy (ACT) and logotherapy. In ACT, values are understood as freely chosen, verbally constructed qualities of ongoing action that guide committed behavior and give direction to psychological flexibility. Viktor E. Frankl, in contrast, distinguished creative, experiential, and attitudinal values, which, in his view, are not invented but discovered in the respective situation. This theoretical paper systematically compares the two conceptions. It identifies convergences, such as the distinction between values and goals, the emphasis on action and engagement, the significance of values in dealing with unavoidable suffering, and the function of values as a counterweight to an exclusively symptom-oriented therapy. At the same time, it highlights fundamental differences that arise from distinct philosophical backgrounds: the functional contextualism of ACT on the one hand and Frankl's phenomenological and personalist anthropology on the other. Particular attention is paid to the question of whether values are chosen or discovered and to the status of attitudinal values. The paper concludes with reflections on how both traditions can learn from each other without blurring their respective premises.

Keywords: acceptance and commitment therapy · values · logotherapy · attitudinal values · psychological flexibility

1 Introduction

In the psychotherapy of recent decades, the concept of values has experienced a remarkable renaissance. A major contribution to this has been made by Acceptance and Commitment Therapy (ACT), which identifies values as one of its six core processes (Hayes et al., 1999, 2012). For readers familiar with logotherapy, this development is of particular interest, because Frankl had already developed, decades earlier, a theory of values that stands at the center of his psychotherapeutic approach (Frankl, 1946).

The obvious assumption that these are two variants of the same basic idea, however, falls short. The two traditions emerged in different epistemological and anthropological contexts, and the same terms sometimes carry different meanings in them. The present contribution compares the two conceptions in order to name commonalities precisely and to avoid prematurely leveling differences.

The comparison proceeds in four steps. First, the ACT concept of values is placed in its theoretical context, and Frankl's theory of values is then outlined. Building on this, convergences and differences are worked out. Finally, the implications for practice, assessment, and research are discussed. The contribution confines itself to the conceptual level and makes no statements about the comparative efficacy of the two approaches.

2 Values in Acceptance and Commitment Therapy

ACT understands itself as an application of functional contextualism and relational frame theory, a behavior-analytic approach to explaining human language and cognition (Hayes et al., 2001). According to this understanding, psychological suffering arises essentially from psychological inflexibility, that is, from the dominance of rigid verbal rules, experientially avoidant strategies, and a narrowing of the behavioral repertoire (Hayes et al., 2006). Psychological flexibility, conversely, is regarded as a fundamental aspect of mental health (Kashdan & Rottenberg, 2010).

In ACT, values are described as freely chosen, verbally constructed consequences of ongoing, evolving patterns of activity that function as reinforcers and give direction to action (Hayes et al., 2012). Essential is the distinction between values and goals: goals can be achieved and completed, whereas values resemble a compass direction by which one orients oneself without ever arriving. Values, moreover, are not supposed to be derived from duty, social pressure, or the wish to avoid pain but to be experienced as personally meaningful. Dedicated instruments have been developed to assess valued action, such as the Valued Living Questionnaire (Wilson et al., 2010).

In therapeutic work in ACT, values are frequently explored through experiential exercises, for instance by imagining one's own funeral or by asking what one wants to stand for in life (Hayes et al., 1999). Values clarification is not a one-time act but an ongoing process closely linked to committed action. At the same time, care is taken not to establish values as new rigid rules that could in turn lead to inflexibility.

3 Frankl's Categories of Values

Frankl distinguished three main ways in which people can realize meaning (Frankl, 1946, 1955). Creative values are realized in what a person gives to the world, for instance through work or creation. Experiential values are realized in what a person receives from the world, for instance in the encounter with nature, art, or another human being. Attitudinal values, finally, are realized in the stance a person takes toward an unalterable fate, in particular toward unavoidable suffering.

For Frankl, meaning and values are not invented but found. According to his understanding, meaning is always related to a specific situation and addresses itself as a call to the individual person, who responds to it in responsibility (Frankl, 1969). Conscience functions here as the "organ of meaning," which can sense the unique meaning of a situation. This conception is embedded in an anthropology that assumes a spiritual dimension of the human being and regards self-transcendence – the orientation toward something or someone beyond oneself – as the hallmark of human existence.

Frankl's categories of values are not to be understood in the sense of a rigid hierarchy; nevertheless, he attributed a special dignity to attitudinal values because they can still be realized even where creative and experiential possibilities are exhausted. Herein lies the point of his thesis of the unconditional meaningfulness of life: as long as a person lives, there remains at least the possibility of taking a stand toward one's fate (Frankl, 1946).

4 Convergences

Despite their different origins, several convergences can be identified. First, both approaches emphasize the distinction between an overarching direction and concrete goals. Frankl's talk of meaning as a task that arises anew in every situation and the ACT metaphor of the compass direction both point to the fact that values cannot be checked off. Second, both traditions place the emphasis on action and commitment rather than on mere insight. Third, both see values as a counterweight to a therapy oriented exclusively toward symptom reduction.

Most striking is their closeness in dealing with unavoidable suffering. ACT invites people not to fight painful inner experiences but to accept them in the service of value-guided action. Frankl's attitudinal values aim at a related stance: where a situation cannot be changed, the freedom to take a stand toward it remains. In both cases, coping with suffering is understood not as its elimination but as a change in one's relationship to it.

5 Differences

The most fundamental difference concerns the status of values. In ACT, values are chosen; their significance results from their function for a person's action, and functional contextualism deliberately refrains from ontological statements about whether values exist independently of human beings (Hayes, 2004). For Frankl, by contrast, values and meaning cannot be chosen arbitrarily but have a demand character that confronts the person from outside. This difference is not merely terminological: it has consequences for the question of whether a therapist may question a client's values, and for the significance of responsibility and conscience.

A second difference concerns attitudinal values. In ACT, acceptance is a process that enables value-guided action but is not itself a value. For Frankl, on the other hand, the stance toward suffering can itself be meaningful – even when no further possibilities for action exist. This position gains particular weight in situations in which the scope for action is severely limited, for instance in serious illness at the end of life.

A third difference concerns the role of self-transcendence. With self-as-context, ACT has a perspective that leads beyond identification with self-

concepts. Frankl's self-transcendence, however, means something else: turning toward a meaning or another person beyond one's own experience. Existential analysis, too, has emphasized personal stance-taking as an independent dimension (Längle, 2013).

A fourth difference concerns the relationship between language and meaning. In ACT, language is regarded as double-edged: the ability to form relational frames makes values and future orientation possible but is at the same time a source of rumination, self-evaluation, and experiential avoidance (Hayes et al., 2001). Frankl, by contrast, considers the spiritual dimension not primarily in terms of its risks but as a source of freedom and responsibility. ACT's skepticism toward the dominance of verbal rules has no direct counterpart in logotherapy, although Frankl's critique of hyperreflection points in a related direction.

6 Conclusions

The comparison shows that ACT and logotherapy are astonishingly close in their practical orientation but diverge clearly in their philosophical foundations. For practice, this can be enriching: ACT offers a multitude of tested exercises for clarifying values and fostering psychological flexibility; logotherapy offers a differentiated language for dealing with unalterable suffering and for the question of what a person orients themselves toward beyond themselves. For research, the task arises of operationalizing the differences in the conceptions of values in such a way that their respective contributions to change become empirically distinguishable. An eclectic blending without reflection on the premises, by contrast, would serve neither tradition.

Different requirements arise from this for measurement. Instruments from the ACT tradition typically assess how important certain life domains are to a person and how consistently they act in accordance with them (Wilson et al., 2010). Logotherapeutic concepts such as the realization of attitudinal values can be represented only inadequately with such instruments, since they concern precisely those situations in which value-consistent action in the usual sense is hardly possible. Here there is a need for further development in assessment.

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REVIEW

Meaning-Centered Psychotherapy in Palliative Care

Conception, State of the Evidence, and Questions of Transferability

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ABSTRACT

In addition to physical symptoms, people with advanced, incurable illnesses frequently face existential distress, such as hopelessness, loss of meaning, or a desire for hastened death. Meaning-centered psychotherapy (MCP), developed by William Breitbart and colleagues, is a manualized intervention that explicitly builds on Viktor E. Frankl's logotherapy and aims to help patients experience meaning even in the face of limited life expectancy. This narrative review first describes the theoretical foundations and the structure of the intervention in its group and individual formats. It then summarizes the evidence from randomized controlled trials, which overall points to favorable effects on spiritual well-being, sense of meaning, and quality of life. Limitations of the existing research are discussed, in particular the concentration on specific populations and health care systems, the handling of attrition in a severely ill population, and questions of cultural transferability. Finally, MCP is compared with other existentially oriented interventions in palliative care, such as dignity therapy. The paper concludes with considerations regarding implementation in German-speaking countries and the qualifications required of clinicians.

Keywords: meaning-centered psychotherapy · palliative care · psycho-oncology · existential distress · logotherapy

1 Introduction

Modern palliative care understands itself as comprehensive support for seriously ill people that takes equal account of the physical, psychological, social, and spiritual dimensions of suffering. Existential burdens at the end of life in particular – the experience of meaninglessness, hopelessness, or the desire for hastened death – pose special challenges for those providing treatment, because they cannot be answered by symptom-oriented measures alone. Yalom (1980) described confronting one's own finitude as a central ultimate concern of existence, which becomes inescapable when facing a life-limiting illness.

Against this background, Meaning-Centered Psychotherapy (MCP) was developed at Memorial Sloan Kettering Cancer Center in New York. It is among the few meaning-centered interventions that have been investigated in several randomized controlled trials and thus represents an important point of reference for the discussion of the evidence base of logotherapeutically inspired procedures. The present contribution provides a narrative overview of its conception and the state of research and asks about its transferability to other care contexts.

Conceptually, existential distress can be distinguished from depressiveness, even though the two may overlap. Hopelessness, loss of meaning, or the feeling of being a burden to others can occur in seriously ill people even without the full picture of a depressive disorder. This gives rise to a need for interventions that explicitly make existential themes their subject.

2 Theoretical Foundations

MCP explicitly draws on the work of Viktor E. Frankl. Its central assumptions are that the striving for meaning represents a fundamental human motivation, that meaning can be experienced even under conditions of severe suffering and limited lifetime, and that people retain the freedom to choose their stance toward unalterable circumstances (Frankl, 1946/2006). Breitbart and Poppito (2014) translate Frankl's categories of values into concrete sources of meaning that are systematically explored over the course of the intervention: historical sources of

meaning in the sense of one's own life legacy, attitudinal sources of meaning in dealing with the limitations of life, creative sources of meaning in the sense of responsibility and commitment, and experiential sources of meaning in love, beauty, and humor.

In addition, MCP can be understood within the framework of Park's (2010) meaning-making model. A life-limiting diagnosis can create a profound discrepancy between global life goals and the current situation. MCP can be understood as structured support for processes of meaning making that do not deny this discrepancy but embed it in a more comprehensive context of meaning.

A further basic idea is the distinction between what cannot be changed in the face of the illness and what remains open to shaping. The intervention invites participants to perceive both: to acknowledge the limits without losing sight of the remaining possibilities. This reflects Frankl's idea that meaning can be realized not only in doing and experiencing but also in one's stance toward the unalterable.

3 Structure of the Intervention

The group format, Meaning-Centered Group Psychotherapy (MCGP), comprises eight weekly sessions, each devoted to a thematic focus (Breitbart & Poppito, 2014). The sessions combine didactic elements, in which concepts of logotherapy are conveyed, with experiential exercises and group discussion. The program begins with an exploration of the concept of meaning and of the question of how the illness has changed the experience of meaning. Sessions on the various sources of meaning follow, with work on one's own legacy playing a particularly important role. The program concludes with reflection on the shared process and a look at hopes for the remaining time.

For patients for whom participation in a group is not possible, an individual format, Individual Meaning-Centered Psychotherapy, was developed, which provides for a shorter sequence of sessions and can be adapted more flexibly to the course of the illness (Breitbart et al., 2012). Manualization facilitates the training of practitioners and the conduct of controlled studies but at the same time requires a high degree of clinical judgment in order to adapt the structure to the individual needs of seriously ill people.

A characteristic element of the intervention is work on a legacy project. Participants are invited to develop a project that expresses their life and values and points beyond their own lifetime – for instance, recording memories, reconciling with a close person, or a creative project. This element connects the creative values described by Frankl with the question of what remains of a life (Breitbart & Poppito, 2014).

4 State of the Evidence

The first randomized pilot study compared MCGP with supportive group psychotherapy in patients with advanced cancer (Breitbart et al., 2010). The results indicated advantages of the meaning-centered condition with regard to spiritual well-being and experienced meaning; in addition, there were indications of a decrease in hopelessness and desire for death. A pilot study of the individual format likewise showed favorable results compared with a control condition (Breitbart et al., 2012).

A larger randomized trial with active supportive group psychotherapy as the comparison condition essentially confirmed this direction of findings (Breitbart et al., 2015). The meaning-centered group showed advantages with regard to spiritual well-being and quality of life, as well as indications of a reduction in depressiveness, hopelessness, and the desire for hastened death. It is noteworthy that the differences emerged relative to an active control condition that itself contained nonspecific factors such as group cohesion and emotional support. Meaning-centered interventions of this kind were also included in the meta-analysis of existential therapies by Vos et al. (2015) and contributed to the overall favorable picture of structured meaning interventions.

At the same time, the limitations of these findings must be borne in mind. The studies were predominantly conducted at a specialized oncology center in the United States. In a seriously ill population, study dropouts due to disease progression or death are unavoidable, which complicates the interpretation of longitudinal data. Finally, it remains open to what extent the results can be transferred to other diagnostic groups, health care systems, and cultural contexts.

A further aspect concerns the choice of outcomes. In addition to symptom measures, the studies used, in particular, instruments on spiritual well-being and quality of life. This choice corresponds to the aim of the intervention but must be taken into account in comparisons with other psychotherapeutic procedures that aim primarily at symptom reduction. The question of which components of the intervention are responsible for the observed changes has also not yet been conclusively answered.

5 Distinction from Related Interventions

Among the existentially oriented interventions in palliative care, Dignity Therapy has become particularly well known alongside MCP (Chochinov et al., 2005). It aims at strengthening the sense of dignity at the end of life and works with a structured interview from which a written document emerges that can be passed on to relatives. Both approaches share an emphasis on legacy and life review. MCP, however, is more psychoeducational in design, comprises several sessions, and pursues the goal of strengthening the experience of meaning with regard to the remaining lifetime as well, whereas Dignity Therapy is more compact and geared toward the creation of a generativity document.

Common to both interventions is that they understand existential themes not as a mere side effect of mental disorders but as an independent subject of palliative care. In this, they differ from a purely psychopathological approach that views existential distress primarily from the standpoint of comorbid depression or anxiety. In practice, the choice between the approaches may depend on the remaining lifetime, physical capacity, and the preferences of the patients.

6 Implications for Care in the German-Speaking Countries

Several questions arise for implementation in the German-speaking countries. First, careful linguistic and cultural adaptation is required that takes religious and ideological diversity into account and does not implicitly presuppose a particular conception of meaning. Second, delivery places high demands on practitioners: in addition to psychotherapeutic competence, familiarity with existential questions, experience in accompanying the dying, and the capacity for self-reflection in dealing with one's own finitude are indispensable. Third, the introduction should be accompanied by an evaluation that captures not only symptom changes but also the experience of meaning, spiritual well-being, and the acceptance of the intervention by patients and relatives.

Overall, MCP can serve as an example of how basic logotherapeutic ideas can be translated into a precisely described, testable intervention. The available findings are encouraging but do not justify generalization beyond the contexts studied. Further independent replications in different health care systems are desirable.

Relatives and bereaved persons also come into consideration as target groups for meaning-centered interventions. Transfer to these groups is conceptually obvious, since they, too, are confronted with loss, farewell, and questions about the meaning of what has happened; the evidence on this, however, is still limited and requires further research.

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PRACTICE AND TEACHING

Logotherapeutically Oriented Counseling for Burnout and Crises of Meaning

Main Features, Procedure, and Limitations – With a Constructed Teaching Vignette

Eleanor M. Whitcombe, Sora Pazer and Samuel R. Ashford
Institute for Meaning-Centered Psychology

ABSTRACT

In the literature, burnout is described as a syndrome of emotional exhaustion, depersonalization or cynicism, and reduced personal accomplishment, and it is classified in ICD-11 as an occupational phenomenon rather than a medical condition. Beyond organizational working conditions, an existential dimension was pointed out early on: burnout can be understood as a consequence of disappointed expectations of meaning in one's work. This contribution on practice and teaching outlines principles of logotherapeutically informed counseling for burnout and crises of meaning. Following a conceptual clarification, central logotherapeutic concepts are made accessible for counseling, including the existential vacuum, the distinction between meaning and success, the three categories of values, and dereflection. The methodological approach is presented in four phases: clarification and differential assessment, relief and stabilization, exploration of meaning and values in Socratic dialogue, and implementation and transfer. An explicitly constructed teaching vignette, which does not represent any real counseling case, serves as an illustration. Particular attention is paid to the boundary between counseling and psychotherapy, especially the need to recognize depressive disorders and other conditions requiring treatment and to refer clients where appropriate, as well as to the risk of individualizing structural stressors.

Keywords: burnout · crisis of meaning · logotherapeutic counseling · Socratic dialogue · counseling psychology

1 Introduction and Conceptual Classification

Since the 1970s, the concept of burnout has become widespread in professional discussion and among the general public. With the Maslach Burnout Inventory, Maslach and Jackson (1981) presented a measurement instrument that captures burnout through the dimensions of emotional exhaustion, depersonalization, and reduced personal accomplishment. In a later review, Maslach et al. (2001) emphasized the importance of the fit between person and work environment, for instance with regard to workload, control, reward, community, fairness, and values. In ICD-11, the World Health Organization lists burnout as an occupational phenomenon in the chapter on factors influencing health status, and explicitly not as a medical condition (World Health Organization, 2019).

The distinction between burnout and depressive disorders is controversial in research and of great importance for practice (Schaufeli & Enzmann, 1998). The present contribution therefore explicitly does not understand counseling as the treatment of mental disorders. It is addressed to professionals in counseling contexts and describes how logotherapeutic concepts can be used in work-related exhaustion and crises of meaning – in awareness of the limits of this framework.

2 The Existential Dimension of Burnout

Pines (1993) interpreted burnout from an existential perspective: people who seek an essential source of meaning in their work are said to be particularly at risk when they experience that their efforts do not have the significance they had hoped for. From this point of view, burnout appears less as the result of mere overload than as the result of disappointed expectations of meaning. This interpretation is related to Frankl's concept of the existential vacuum, which describes a feeling of inner emptiness and meaninglessness (Frankl, 1969).

Also significant from a logotherapeutic standpoint is Frankl's distinction between meaning and success. Anyone who measures the value of their own work exclusively by measurable results, recognition, or efficiency makes their experience of meaning dependent on conditions that frequently lie outside their own control. Existential analysis has differentiated this further and has pointed to the importance of inner consent to one's own activity (Längle, 2013). According to this understanding, activity that is successful but carried out without inner consent can be exhausting in the long run.

Nevertheless, caution is called for: an existential interpretation must not lead to individualizing structural causes such as work intensification, a lack of resources, or unfair organizational conditions. Meaning-centered counseling does not replace a change in stressful working conditions; at most, it can support the person in recognizing and using their scope for action.

3 Logotherapeutic Concepts for Counseling

Several logotherapeutic concepts are particularly helpful for counseling in burnout and crises of meaning. Frankl's three categories of values (Frankl, 1946) open up a differentiated view of sources of meaning: creative values are realized in work itself but are not restricted to it; experiential values in relationships, nature, or culture can form a counter-movement to a way of life one-sidedly geared toward performance; attitudinal values concern the stance toward burdens that cannot be changed immediately. Schnell (2020) has also pointed out that a greater diversity of sources of meaning may be associated with a more stable experience of meaning.

A further concept is dereflection. It aims at dissolving excessive, worried self-observation by directing attention to meaningful tasks and relationships beyond one's own state of mind. Lukas (1998) described in detail the methodical application of dereflection and other logotherapeutic procedures for counseling and therapy. In exhaustion, however, dereflection must be used cautiously: it must not be misunderstood as an invitation to ignore the body's warning signals.

It is also helpful to ask whether meaning is derived exclusively from work or whether other areas of life contribute to the experience of meaning. People with very high occupational identification often derive a large part of their experience of meaning from work. If this source falls into crisis, alternatives are lacking. Counseling can help to broaden the spectrum of possible sources of meaning again without devaluing the importance of work.

4 Methodical Procedure in Four Phases

The first phase serves clarification and delineation. Here, concerns, stress factors, and resources are assessed. At the same time, it must be carefully examined whether there are indications of a depressive disorder, a substance use problem, suicidality, or physical illnesses that require medical or psychotherapeutic evaluation. In these cases, counseling cannot replace treatment and should work toward an appropriate referral.

The second phase aims at relief and stabilization. Recovery, sleep, social support, and, where appropriate, conversations with supervisors or workplace contact persons are in the foreground. In this phase, questions of meaning are acknowledged rather than explored in depth, since an acutely exhausted person often lacks the strength for a fundamental reorientation.

In the third phase, meaning and values are explored, preferably in Socratic dialogue. Understood logotherapeutically, this does not aim at ascribing meaning to the person seeking advice but at enabling them, through questions, to discover their own experiences of value and their own possibilities. Here there are points of contact with guided discovery in cognitive behavioral therapy, which likewise works with systematic questioning (Kazantzis et al., 2014; Overholser, 1993). The difference lies in the subject matter: whereas Socratic dialogue in cognitive behavioral therapy serves primarily to examine beliefs, in logotherapy it is directed toward values, responsibility, and possibilities of meaning.

The fourth phase serves implementation and transfer. Together, concrete steps are developed that carry what has been recognized over into everyday life: a changed weighting of areas of life, clarification of professional roles, cultivation of neglected experiential values, or even the decision for a professional reorientation.

It is important that the steps remain realistic and do not intensify the exhaustion through new performance demands. Logotherapeutically oriented counseling asks not only what a person should do but also what they may let go of and what stance they wish to take toward what cannot be changed for the time being.

5 Constructed Teaching Vignette

Note: The following vignette is a presentation constructed for teaching purposes. It does not reproduce a real counseling case; any resemblance to actual persons is unintended.

An experienced nurse on an internal medicine ward seeks out a counseling center because she has been feeling exhausted, irritable, and inwardly empty for months. She reports that she used to experience her profession as a vocation but now has the impression that she is "just functioning." In the first phase, the counselor clarifies whether there are indications of a depressive disorder and recommends an evaluation by a general practitioner, which the client also follows up on. In the stabilization phase, possibilities for relief are discussed, including a conversation with the ward manager about staff scheduling.

In the exploration phase, the counselor asks in Socratic style in which moments at work the client most recently had the impression that what she was doing mattered. The client remembers a conversation with a seriously ill patient for which she had taken a few minutes despite time pressure. In the further dialogue, it becomes clear that what is meaningful to her is less the number of tasks completed than the quality of the encounter, and that it is precisely this that falls short under the current conditions. In the transfer phase, she develops concrete steps to shape such encounters more consciously and at the same time decides to raise the structural burdens within the team.

For teaching, the vignette illustrates three points: the necessity of careful clarification at the outset, the combination of individual exploration of meaning with naming structural burdens, and the function of Socratic dialogue as a method of discovery rather than instruction.

6 Limitations and Conclusions

Logotherapeutically oriented counseling can offer people with work-related exhaustion and crises of meaning a differentiated framework for understanding their situation and discovering possibilities for action. Its efficacy in the context of burnout, however, has not yet been demonstrated in controlled studies; the main features presented here are therefore to be understood as a theoretically grounded and practice-oriented guide, not as an evidence-based treatment program. For continuing education, the task arises of strengthening, in addition to conveying logotherapeutic concepts, in particular the competencies for distinguishing conditions requiring treatment and for reflecting on organizational framework conditions.

For teaching, the procedure described is particularly suitable for practicing a questioning rather than an instructive style of conversation. In role-plays, students can try out how questions about experiences of value can be formulated without suggesting answers, and how indications of conditions requiring treatment can be recognized in conversation. Supervision and self-reflection are indispensable here, because work on questions of meaning touches on the counselors' own values, and there is a risk of transferring these unnoticed.

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BOOK REVIEW

A Psychologist Experiences the Concentration Camp

*Viktor E. Frankl's *... trotzdem Ja zum Leben sagen* or *Man's Search for Meaning* – A Rereading from Today's Perspective*

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ABSTRACT

Viktor E. Frankl's account of his experiences in National Socialist concentration camps was first published in Vienna in 1946 under the title "Ein Psychologe erlebt das Konzentrationslager" and later became known as "... trotzdem Ja zum Leben sagen." In its English version, "Man's Search for Meaning," which was expanded by an introduction to logotherapy, the book is among the most widely read psychological works of the twentieth century. This review undertakes a rereading from the perspective of contemporary psychology. It first describes the structure and content of the book, in particular Frankl's account of the prisoners' psychological reactions in three phases and his reflections on inner freedom and meaning in extreme suffering. It then appraises the book's particular genre, situated between testimony, phenomenological description, and programmatic foundation of logotherapy. Critical points include the limits of generalizing from individual experience, the risk of retrospectively attributing meaning to survival, and the relationship of the book to other survivor testimonies. The review concludes that the work retains its significance as an existential document and as a point of departure for meaning-centered psychology, but should not be read as empirical evidence for the effectiveness of meaning orientation.

Keywords: Viktor E. Frankl · book review · concentration camps · logotherapy · inner freedom

1 The Work Under Review

Reviewed: Frankl, V. E. (1946). Ein Psychologe erlebt das Konzentrationslager. Verlag für Jugend und Volk; and the English-language edition Frankl, V. E. (2006). Man's search for meaning. Beacon Press. Since the late 1940s, the German-language version has appeared in numerous editions under the title *... trotzdem Ja zum Leben sagen* ("... Saying Yes to Life in Spite of Everything").

Hardly any psychological book of the 20th century has found a comparable readership. By his own account, Frankl wrote the report within a few days of his return to Vienna, and it was originally meant to be published anonymously. That a scholarly journal should review this book some eight decades after its first publication requires justification: it is not only a historical document but, to this day, the first point of access to logotherapy for many people, and thus a text whose readings shape the understanding of meaning-centered psychology.

The review follows the original German-language version and draws on the English-language edition insofar as it contains additional parts. Content is rendered in paraphrase; extensive verbatim excerpts are avoided.

2 Structure and Content

The core of the book is Frankl's account of his experiences as a prisoner. In 1942, Frankl was deported with his family to the Theresienstadt ghetto and later taken to Auschwitz and to subcamps of the Dachau concentration camp; his parents, his brother, and his first wife perished (Klingberg, 2001; Längle, 1998). Frankl largely dispenses with a chronological narrative and instead attempts to describe typical psychological reactions of the prisoners. He distinguishes three phases: the shock of admission; the phase of camp life proper, characterized by apathy, emotional blunting, and reduction to immediate survival; and the phase after liberation with its own

difficulties, including feelings of depersonalization and bitterness (Frankl, 1946/2006).

Running through the description are reflections that later become basic ideas of logotherapy. Frankl emphasizes that even under extreme conditions a last freedom remains to the human being: the freedom to choose one's attitude toward the circumstances. He reports moments of inner turning toward loved ones, the significance of a task that awaits one, and fellow prisoners who gave others their last piece of bread. At the same time, he describes how the loss of any perspective on the future could undermine inner stability.

The English-language edition supplements the account with a brief introduction to the basic concepts of logotherapy and a later postscript in which Frankl sets out his position of "tragic optimism": the conviction that people can find meaning even in the face of suffering, guilt, and death (Frankl, 1946/2006).

Stylistically remarkable is the restraint with which Frankl speaks of himself. He usually describes his own experiences only insofar as they illustrate typical experiences and refrains from a detailed account of his own suffering. This restraint lends the text a peculiar matter-of-factness that many readers find particularly compelling.

The logotherapeutic part of the English-language edition is kept brief. It introduces concepts such as the will to meaning, the existential vacuum, noogenic neurosis, and paradoxical intention. Anyone wishing to become systematically acquainted with logotherapy must turn to Frankl's major professional works; as an introduction to Frankl's thought, however, this part serves its purpose.

3 Appraisal

The particular achievement of the book lies in its combination of testimony and psychological observation. Frankl writes as someone affected, but with the eye of a psychiatrist, and he strives for a language that does not accuse but seeks to understand. The depiction of psychological reactions to extreme stress is impressive in its precision, even if it cannot claim to be a systematic investigation.

For psychotherapy, what is significant above all is Frankl's insistence on a dimension of human freedom that persists even where external freedom is completely lacking. This idea is central to the later conception of attitudinal values and has influenced existentially oriented approaches far beyond logotherapy (Yalom, 1980). Contemporary research on meaning also frequently refers to the book as a starting point, although it now develops its concepts independently and tests them empirically (Wong, 2012).

Also noteworthy is the importance Frankl attaches to future orientation. He describes how fellow prisoners who had before their eyes a task or a person for whom they wanted to go on living found inner stability, and in this context he takes up Nietzsche's dictum that he who has a why to live can bear almost any how. These observations anticipate questions that were taken up in later research on hope, goal orientation, and meaning making under stress.

Didactically, the book is excellently suited as an entry point for courses on existential psychotherapy. It stimulates engagement with questions that are easily neglected in a curriculum focused on disorders and techniques: What remains to a person when almost everything has been taken from them outwardly? What role do attitude, relationship, and responsibility play in coping with extreme stress? In a seminar, such questions can be productively linked with contemporary research on meaning.

Not least, the book impresses with its linguistic clarity. Frankl largely avoids technical terminology and thereby reaches a readership far beyond the professional world. That a text matured under the most difficult conditions imaginable manages without pathos and nevertheless leads to profound questions is among the reasons for its lasting impact.

4 Critical Remarks

A rereading from today's perspective must at the same time point out limitations. First, the book is based on the experience and memory of a single individual. Frankl's descriptions of typical reactions are generalizations from personal observation; they can provide valuable hypotheses but cannot ground empirically established statements about the psychological consequences of extreme traumatization.

Second, there is the danger of a reading that attributes survival to an inner orientation toward meaning and thereby, conversely, suggests that those who were murdered lacked meaning or inner stance. Frankl himself contradicted this interpretation and emphasized that the best of us did not return (Frankl, 1946/2006). In his later reflections, Primo Levi forcefully highlighted the role of chance and external circumstances in survival (Levi, 1986). A responsible reading should include this perspective so as not to

inadvertently establish a hierarchy between survivors and the murdered.

Third, the book should be read in dialogue with other testimonies. The accounts of Levi (1958) and the essays of Améry (1966) set different emphases and reveal that the experience of the camps eludes any unified interpretation. Precisely in contrast to them, it becomes apparent that Frankl's account is not only testimony but also the foundation of a therapeutic program – a dual function that should be kept consciously in mind when reading.

Fourth, attention must be drawn to the history of the book's reception. It is frequently quoted in self-help and motivational contexts in which individual sentences are detached from their context. Such a reception threatens to obscure the gravity of what is reported and to reduce Frankl's ideas to a formula of positive thinking. Frankl himself opposed any glorification of suffering and emphasized that avoidable suffering is to be eliminated.

Finally, it would be desirable to bring the psychological consequences of extreme traumatization that Frankl hints at for the period after liberation into closer dialogue with later psychotraumatological research. Frankl's descriptions of depersonalization, bitterness, and disillusionment after the return are among the less noticed, but psychologically particularly illuminating, passages of the book.

5 Conclusion

... trotzdem Ja zum Leben sagen (Man's Search for Meaning) remains a significant document of human self-assertion under extreme conditions and an indispensable starting point for understanding logotherapy. The book can be recommended without reservation for university study and continuing education, provided that it is read not as empirical proof of the efficacy of meaning orientation but as existential testimony and as a source of theoretical intuitions. In addition, readers should consult biographical accounts (Klingberg, 2001; Längle, 1998) as well as other testimonies of survivors in order to grasp the historical and human dimension of the text adequately.

For the Journal of Meaning-Centered Psychology, this rereading is at the same time a form of self-reassurance: it reminds us that the scientific study of meaning emerged from an existential experience that eludes any instrumentalization. Methodological rigor and respect for this origin are not mutually exclusive but mutually dependent.

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